

Missouri Transformation of Rural Community Health Care (ToRCH Care) Frequently Asked Questions (FAQ)

The Rural Health Transformation Program information provided by the Missouri Department of Social Services is supported by the Centers for Medicare & Medicaid Services (CMS) of the U.S. Department of Health and Human Services (HHS) as part of a financial assistance award totaling \$216,276,817.66, with 100 percent funded by CMS/HHS. The contents are those of the author(s) and do not necessarily represent the official views of, nor an endorsement by, CMS/HHS, or the U.S. Government.

Official Links

MO RHTP website for program updates	https://mydss.mo.gov/mhd/rural-health
CMS RHTP website	CMS RHTP program overview
MO DSS bid proposals website	MO DSS bid proposals
Official Hub boundary map	Hub boundary press release
Hub Anchor application	Hub Anchor request for application press release
Webinar slides and recordings	ToRCH Care overview webinar Hospital-specific program deep dive Non-hospital clinical providers deep dive Community institutions deep dive Hub Anchor application deep dive
Program email	DSS.RHTP@dss.mo.gov

FAQ Contents

Section	Page
RHTP and ToRCH Care overview	2
Hub, RCN, and participation overview	3
Hub Anchors and Hub governance overview	6
Funding and ToRCH Care programs overview	9
Data and technology overview	10
Next steps	12

RHTP and ToRCH Care overview

Q1. What is the Rural Health Transformation Program (RHTP)?

The federal RHTP is a \$50 billion five-year program meant to build a stronger, more sustainable healthcare system for the future. The Missouri Department of Social Services, through the MO HealthNet Division and the Rural Health Transformation Office (RHTO), is implementing the RHTP for Missouri to build a stronger, more sustainable healthcare system for the future.

Q2. What is ToRCH Care?

The transformation of rural community health care (ToRCH Care) model is Missouri's approach to implementing the RHTP priorities. ToRCH Care is designed to preserve and strengthen access to care in rural communities through coordinated investments in healthcare provider workforce, digital infrastructure, payment reform, provider transformation, and the activation of Regional Coordinating Networks (RCNs) and Hubs. It is built on the success of the ToRCH pilot program that demonstrated clear improvements in healthcare outcomes, including a 21% reduction in avoidable inpatient stays.

The innovative ToRCH Care proposal resulted in Missouri being awarded \$216M in year one - the ninth most funding out of all fifty states.

Q3. What is the overall ToRCH Care strategy?

The foundation of the ToRCH Care model is a statewide network of local community "Hubs" made up of 2-6 neighboring rural counties. The model is designed to bring care closer to home, improve local health outcomes, and ensure care coordination across a wide range of local providers.

Q4. What will ToRCH Care do for Missourians?

RHTP offers Missouri an unprecedented opportunity to confront longstanding challenges for the approximately 2.5 million Missourians in rural areas facing widening health disparities. The ToRCH Care proposal intends to address the chronic access and outcome disparities for those rural residents. The detailed key performance metrics across access, outcome, and sustainability measures can be found in the [Anchor application](#).

Q5. What are the main elements of ToRCH Care?

ToRCH Care is built around five core initiatives:

- Regional Coordinating Networks and Hub Activation: Create local and regional groups that help providers coordinate care and focus on critical needs, spanning 104 counties
- Alternative Payment Models: Implement new payment approaches for providers that reward teamwork and better health outcomes
- Digital Backbone: Build resilient, modern, and shared technology systems so providers can communicate and share information easily
- Rural Health Workforce Programs: Train, recruit, and keep healthcare workers in rural areas to provide access to care
- Provider Transformation: Help rural hospitals, care centers, and clinics ensure core access preservation and modernize operations

Q6. Who helped shape Missouri's ToRCH Care program?

A broad coalition of stakeholders provided input in shaping the program and its key initiatives, including state agencies, various professional associations, local public health agencies (LPHAs), critical access hospitals (CAHs), federally qualified health centers (FQHCs), rural health clinics (RHCs), emergency medical services (EMS) agencies, community-based organizations (CBOs), and public input from rural residents, patients, and community leaders across rural counties.

Q7. Where can organizations find updates, webinar recordings, application materials, or procurement information?

Organizations can use the RHTP landing page at the top of the FAQ document to access program updates, application materials, procurement information, webinar recordings, and more.

Organizations may also [subscribe](#) to MO DSS or contact DSS.RHTP@dss.mo.gov for program updates.

Q8. What can and can't RHTP funding be used for?

The RHTP program empowers states to strengthen rural communities through funding areas such as:

- Evidence-based interventions for preventive care and chronic disease management
- Training and assistance in bringing technology and tech-enabled solutions to people and providers, including for efficiency and care delivery
- Workforce development programs to bring healthcare workers to rural areas
- Innovative care model development, like alternative payment models

Note, CMS also outlines multiple areas where funding is not permitted including supplanting federal funding, duplicating existing services, significant new construction, lobbying activities, and purchasing food products. Please see the CMS overview link at the beginning of this FAQ for further details.

Q9. Does RHTP funding pay for direct patient care and services?

No. RHTP funding is intended to transform care delivery, expand access, strengthen coordination, and support sustainable systems. RHTP funding should not duplicate billable services or replace payment for services that are already reimbursable by insurance.

Hub, RCN, and participation overview

Q10. What is a ToRCH Care Hub?

A Hub is a community-based network of local partners coordinating care in a rural community. It is a formal partnership, led by a Hub Anchor, among local clinical, public health, EMS, pharmacy, non-clinical, and community-based partners, as well as other engaged local stakeholders. Within a Hub, partners collaborate to understand and track local needs, coordinate care through connected referral systems, and implement programs targeted to improve community-specific needs. Hubs are not a new physical entity or location, and they do not act as a fiscal agent.

Q11. Will Hubs limit residents' ability to receive care outside of their Hub?

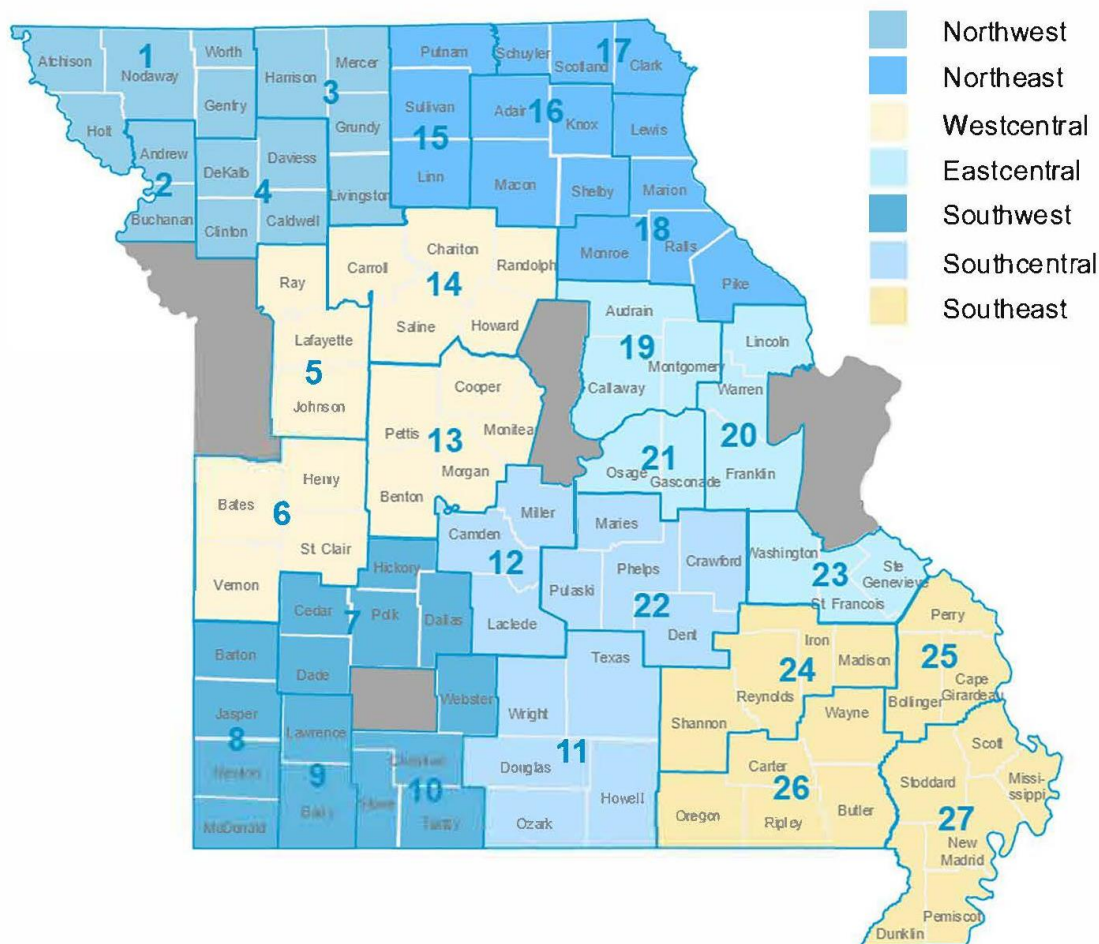
No. Hubs will not prevent residents from using services provided by organizations outside of the Hub they live in, nor will they prevent cross-Hub collaboration. Hub residents will still have access to providers they regularly use regardless of Hub boundaries, and Hubs will be actively encouraged to coordinate and learn from each other.

Q12. If a patient receives care outside their Hub, will insurance stop paying?

No. A Hub is not an insurance network. The Hub does not replace, limit, or restrict existing services; it connects and amplifies them. Patients and members should continue to follow the coverage, network, and authorization rules of their health plan.

Q13. What is a Regional Coordinating Network (RCN)?

To build the central foundation of ToRCH Care, seven Regional Coordinating Networks (RCNs) connect and provide oversight to regional groupings of Hubs across the state. RCNs support Hubs by providing technical assistance, analytic support, and shared learning across regions. RCNs also help align Hub proposals with state guardrails, support contracting programs, and monitor vendor compliance where applicable. Together, RCNs and Hubs form an integrated operating model that enables coordinated and improved care across providers with the ability to track outcomes and impact.



Q14. How are the RCNs structured?

RCNs will operate under an overarching regional management organization that will support contracting for programs, with money flowing through it, and will provide regional technical assistance to support providers and partners. RCNs will receive dedicated staffing to support ToRCH Care integration, including a regional director for each region, technical assistance specialists, data & analytics managers, community partnership liaisons, clinical leads, and fiscal managers.

Q15. Are RCNs, Hubs, and Hub Anchors the same thing?

No. These are distinct parts of the ToRCH Care system. RCNs provide regional technical assistance, oversight of Hubs, approval of proposed local programs, analytics, and contracting support. Hubs are local partnerships made up of providers and community partners. A Hub Anchor is one organization within a Hub that serves as the lead convener and coordinating organization.

Q16. What is the role of the Rural Health Transformation Office (RHTO) in Hubs?

The Rural Health Transformation Office (RHTO) provides statewide oversight, strategic guidance, technical support, and issue resolution for all local community Hubs and RCNs. Its role is to ensure compliance to CMS requirements and oversee the successful implementation of ToRCH Care in Missouri.

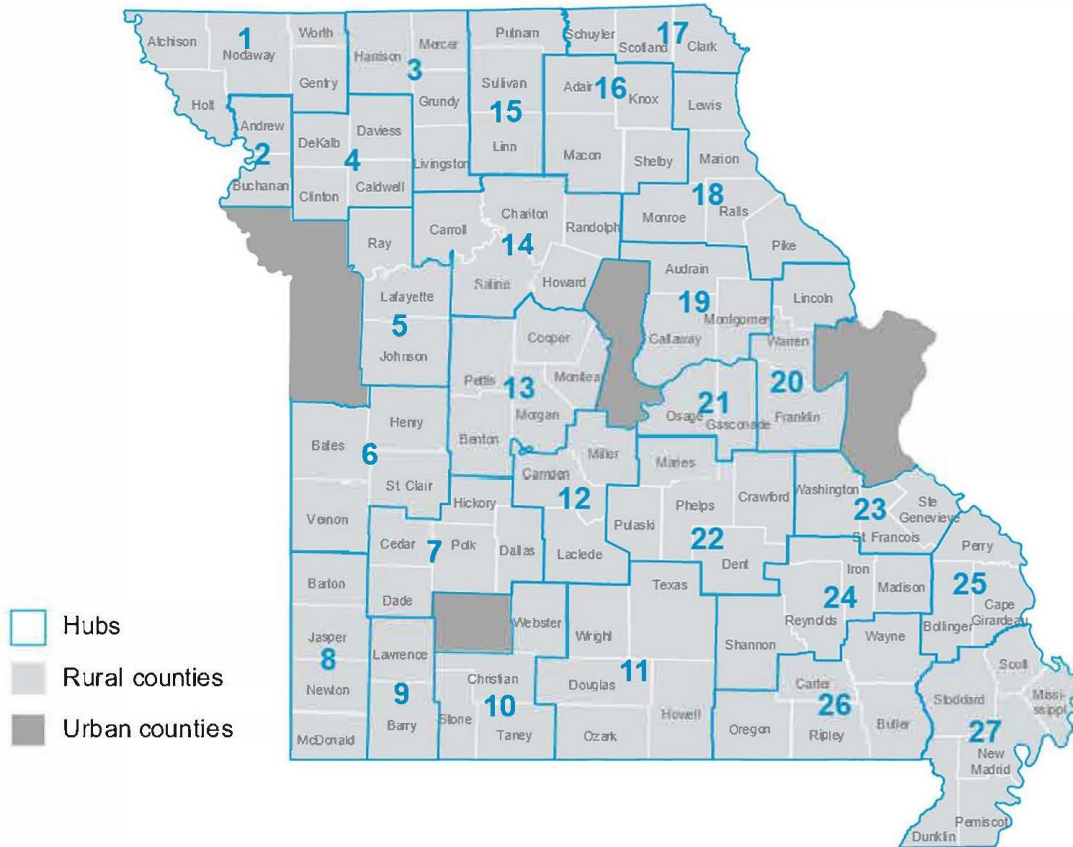
Q17. How will statewide, RCN, and Hub contracting work?

There will be different contracting pathways depending on the program. The Healthier Communities Together entity will support contracting for local Hub programs, with money flowing through it, while the state will manage select statewide contracts.

Hubs will work with RCNs to direct funds and select organizations may apply to receive additional funding through program-specific opportunities, however, Hubs do not act as a fiscal entity in any capacity.

Q18. What are the official Hub boundaries?

There are 27 Hubs across the state spanning 104 rural counties. Each Hub is comprised of 2-6 counties serving populations ranging from 15k to 217k. The official Hub boundary map is below:



Once an organization identifies its Hub, local providers and organizations in that area should convene to align on Hub participation and the Hub Anchor application. For further details see the official Hub boundary press release linked at the top of the document.

Q19. How were boundaries drawn?

The Hub boundary map reflects several factors, including:

- Comprehensive stakeholder feedback, including ~500 comments from the public and members of Missouri's healthcare ecosystem.
- Rural population, density, and other demographics
- Provider locations and access considerations

Q20. What are the potential roles of Hub partners?

There are two distinct roles for providers and community partners to play with distinct responsibilities (non-exhaustive):

- Hub partners will collaborate with other providers to coordinate care and improve local health outcomes, participating in referral networks and leadership board meetings.
- Hub Anchors will serve as the convener of all partners and the connection to the broader RHTO infrastructure.

Q21. Who can participate as a Hub partner?

All interested local organizations including clinical and non-clinical partners can participate as Hub partners! Referrals, leadership board meetings, and other activities are intended to span the entire healthcare ecosystem.

Organizations may include but are not limited to: Hospitals, Federally Qualified Health Centers (FQHCs), Rural Health Clinics (RHCs), behavioral health providers, Emergency Medical Services / Mobile Integrated Healthcare, pharmacies, Local Public Health Agencies (LPHAs), other primary care clinics, non-clinical providers, community-based organizations (CBOs), and other social entities such as faith, fire, schools, and community groups.

Q22. How are local priorities selected?

During the Hub Anchor application period, applicants are required to coordinate with local providers to identify local health needs with data-based support, such as existing Community Health Needs Assessments.

Once the official Anchors are selected and leadership boards established, Hubs will use this information to collaboratively propose programs targeted to local populations and local access or outcome gaps, while operating within statewide guardrails. Each Hub will look different based on which organizations are active in it and what local programs are prioritized. Final proposals for programs will be approved by the Hub Leadership Board and submitted to RCNs and the RHTO for final approval.

Q23. What roles can community institutions play in a Hub?

Community institutions play an integral role to help identify food, housing, transportation, safety, and other non-clinical needs; connect residents to services through shared workflows; participate in leadership boards or working groups; receive and respond to referrals; and document actions and outcomes so the Hub can monitor progress.

Q24. Do organizations need to be Hub Anchors to participate?

No. Not every organization will want or need to serve as an Anchor. Many organizations will participate as Hub partners, leadership board members, referral partners, or program implementers.

Q25. Does a Hub need a physical location or office?

No. A Hub is not a new building or a single clinic. It is a formal partnership of local healthcare-related organizations led by a Hub Anchor.

Q26. How should vendors or service providers engage?

Vendors interested in engaging with planned RHTP funding areas should monitor the RHTP and the MOBuys website for procurement opportunities. At the local and regional levels, providers and community partners can engage through Hub convening, leadership board discussions, and referral or program workflows where appropriate.

Hub Anchors and Hub governance overview**Q27. What is a Hub Anchor?**

Hub Anchors are the neutral coordinator of organizations within a Hub, driving the collective efforts of all participants to improve local patient outcomes. The Anchor is responsible for bringing partners together through a leadership board, coordinating referral workflows, helping implement local programs, and using dashboards and reports to send data back to the state.

Q28. Does the Hub Anchor own or direct other providers?

No. The Hub model is designed to operate through partnership. The Anchor remains a participating organization within the Hub and does not vertically integrate or direct other providers. Decision-making is shared and grounded in local input.

Q29. Who makes decisions for the Hub?

Decisions are made through the leadership board through a voting process, not by the Hub Anchor alone. The Hub Anchor facilitates and documents decisions. The leadership board and partner organizations, including the Hub Anchor, help drive and implement strategies and programs.

Q30. What is a Hub leadership board?

The Hub leadership board brings together leaders from clinical and non-clinical partners across the healthcare ecosystem to drive progress towards supporting residents. Leadership board members review data on local needs and outcomes, vote on which issues to prioritize according to community needs, provide strategic direction and governance, and oversee Hub performance and progress.

Q31. How are local priorities selected?

During the Hub Anchor application period, applicants are required to coordinate with local providers to identify local health needs with data-based support, such as existing Community Health Needs Assessments.

Hubs will then convene within their leadership boards and use this information to propose programs targeted to local populations and local access or outcome gaps, while operating within statewide guardrails. Each Hub will look different based on which organizations are active in it and what local programs are prioritized.

Q32. What types of organizations are eligible to apply to be a Hub Anchor?

Any type of organization is eligible to apply to be a Hub Anchor, and partners within Hubs are encouraged to collaboratively align on who is best suited to apply.

Q33. How will Hub Anchors be evaluated and selected?

Selection is based on the criteria outlined in the Hub Anchor application, including ToRCH Care alignment, governance, partnership depth, staffing readiness, and digital readiness.

Organizations are encouraged to collaboratively align on a Hub Anchor organization and complete the application together.

Q34. What partner support is expected in the Hub Anchor application?

Applicants must demonstrate meaningful community engagement before applying. Letters of support should be included where possible from at least one clinical provider, one pharmacy, one EMS / MIH-CP provider, one Local Public Health Agency, and one CBO. Additional letters of support may also be provided.

Letters of support should describe the organization's anticipated role in the Hub, willingness to participate in the Leadership board, data sharing, and workflows, and the organization's own perspective and commitment to the Hub collaboration.

Q35. What is the Hub Anchor application timeline?

The Hub Anchor application's submission deadline is June 18, 2026 at 11:59 PM CT.

Q36. Will Hub Anchors receive funding or compensation?

Hub Anchors will not act as the fiscal agent. The regional management organization will support contracting for programs to centralize functions such as contracting and compliance across Hubs.

The State will fund additional staff to support implementation via the Healthier Communities Together entity. Funding availability, eligibility, and allowable uses may vary by program, procurement approach, and Hub priorities.

Q37. What staff will be provided to support Anchors or other Hub partners?

Across the 27 Hubs, Program Coordinators and RHTP Navigators will support local implementation.

Every Hub Anchor will be allocated a program coordinator to prevent overextension of existing roles and centralize coordination functions.

During the application period, Hubs will work to select priorities for outcome improvements and based on those needs will decide where RHTP Navigators sit. Each Hub has been allocated a certain number of RHTP Navigators according to Medicaid populations. Please see the Hub Anchor application for further details.

Q38. What are the roles and responsibilities of Program Coordinators?

Program Coordinators are responsible for organizing, implementing, and managing day-to-day program operations. Their responsibilities include:

- Convening and leading Hub partners in the leadership board
- Serving as the primary Hub spokesperson
- Managing implementation of localized programs
- Monitoring Hub progress and performance
- Supporting local CBOs in implementing referral pathways

Q39. What are the roles and responsibilities of RHTP Navigators?

RHTP Navigators serve as public health professionals who act as a liaison between residents and health/social services. Their responsibilities include:

- Serving as an integrator of clinical, behavioral, and social care referrals
- Providing proactive, relationship-based engagement with patients
- Supporting providers and community partners across the Hub through operational insights
- Sharing outcome data, qualitative insights, and best practices with Hub partners and the RCN

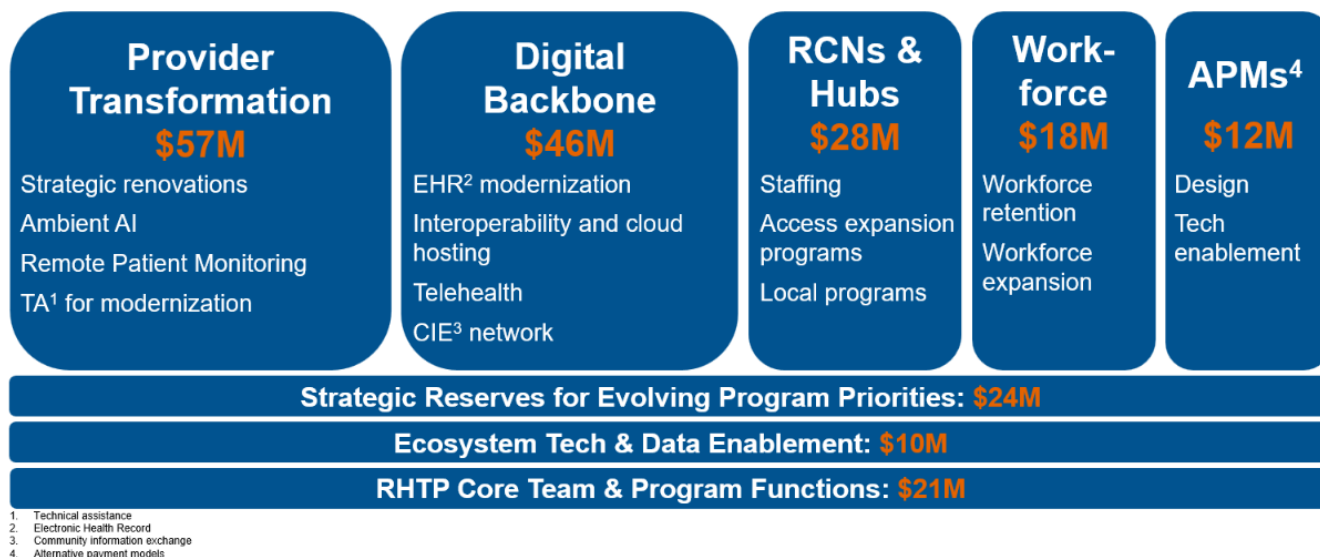
Q40. Who will employ the Hub Program Coordinator and RHTP Navigators?

Program Coordinators will be employed by the Hub Anchor, while RHTP Navigators will be employed by the Hub Anchor or another Hub partner based on local needs. The employing organization will be responsible for recruiting, selecting, and overseeing staff, while the overarching regional management organization will be responsible for reimbursing the employing organization for staffing costs.

ToRCH Care programs overview

Q41: What programs are supported in year 1 and how much funding do they get?

As part of the \$216M awarded to Missouri, the majority (~90%) has been directed into the rural healthcare ecosystem as seen below:



Q42. What level of control will Hubs have over the “local programs”?

Hubs may implement a portfolio of shared programs addressing high need local health outcomes. Hubs should demonstrate how proposed programs target local residents’ needs and alignment with ToRCH Care goals. Example programs include supporting school-based clinics, nutrition focused programming, non-emergency medical transportation and ride coordination.

Note, the federal RHTP prohibits significant new construction, supplanting federal funding, duplicating existing services, lobbying activities, and purchasing food products. Full restrictions on funding can be found on the CMS website.

Q43. How will schools / higher education fit into ToRCH Care?

Schools will play an integral role in ToRCH Care, participating in multiple workforce training and expansion programs, and in Hub leadership boards. Workforce programs include medical school clerkship expansion, maternal health workforce expansion, and other early healthcare workforce pathways.

Q44. How will Mobile Integrated Healthcare fit into ToRCH Care?

As part of Hub/RCN activation, dedicated funding is planned for implementing and expanding Mobile Integrated Healthcare / Community Paramedicine services by supporting paramedic staffing time for non-emergency, community-based care, including post-discharge follow-up and chronic disease stabilization. This will be managed at the RHTO level, and more information will be made available later in the year.

Q45. Will there be behavioral health-specific opportunities?

Yes. Behavioral health focused programs are integrated into multiple initiatives, including training programs and support of specialty consultation lines. Behavioral health and substance use disorder providers are encouraged to participate in Hub leadership boards and referral flows.

Q46. How does telehealth fit into the program?

Telehealth is included in digital backbone funding, intended to expand access to care by integrating virtual care and specialty support into existing local workflows. Telehealth solutions will work to improve rural healthcare

delivery by addressing geographic, workforce, and access barriers to ensure timely, high-quality care close to home. Telehealth licenses will only be available to rural providers. Additional detail will be released when available.

Q47. How does Remote Patient Monitoring fit into the program?

Remote Patient Monitoring (RPM) is included in provider transformation funding intended to improve chronic disease management, maternal health outcomes, reduce hospital readmissions, and enhance patient engagement through technology enabled care.

RPM implementation support will include deploying devices, providing integration and deployment support, and delivering structured technical assistance. Additional detail will be released when available.

Q48. How does ToRCH Care support aging populations?

Through improved care coordination and closed loop referrals, aging populations will receive a more seamless healthcare experience. In addition, targeted access and chronic disease management programs will support aging populations facing access and care disparities.

Hubs with high proportions of aging populations may also provide targeted support through community paramedicine, telehealth, remote monitoring, transportation, and other locally selected programs.

Q49. What does “Alternative Payment Model” mean?

An Alternative Payment Model, or APM, is a type of payment approach that rewards quality and value-based care. Specific models can vary, but it is an alternative reimbursement approach to the traditional volume-based fee-for-service model that can be challenging for rural providers.

The APM pillar is intended to help providers succeed with future value-based care models and create a path to long-term financial stability with shared savings from improved outcomes.

Q50. How do Medicaid MCOs, Medicare Advantage, and commercial payers fit into APMs?

As part of ToRCH Care, the State hopes to stimulate greater adoption of Alternative Payment Models (across Medicaid, Medicare, Medicare Advantage, and Commercial populations) that support improvements in outcomes at a lower cost. In the coming weeks and months, the RHTO will gather input from Missouri providers as well as Managed Care Organizations on their experience with existing APMs, and what types of changes to APMs could support greater adoption and success.

Q51. Can strategic renovation funding include equipment replacement?

Please see the official application and program materials for details on strategic renovation funding on the Missouri DSS bid proposals page at the top of the FAQ, including the requirements for each funding opportunity.

Q52. Can organizations outside Missouri participate or apply?

The RHTP award is made to the State of Missouri and is focused on rural Missouri communities. Specific procurement or funding opportunities will detail eligibility requirements. Out-of-state vendors or organizations should follow official procurement announcements and state guidance.

Data and technology overview

Q53. What is the Digital Backbone?

The Digital Backbone pillar is a shared technology and data foundation that connects rural providers, systems, and community services to enable coordinated, data-driven care.

It will support rural EHR modernization and interoperability, adoption through training and technical assistance, data and AI governance, privacy, security, responsible-use standards, and a social care referral platform.

Q54. What is a closed-loop referral?

A closed-loop referral means a referral is sent to a community organization, the organization confirms they received it and provides the service, and those updates are shared back with the person who made the referral. The social care referral platform will let users send, receive, and track referrals for services like food, housing, transportation, and utility support, so everyone can see what's happening and make sure people—especially in rural communities—get the help they need without things falling through the cracks.

Q55. Does closed-loop referral mean referrals stay inside one health system?

No. Closed-loop referrals support coordination across healthcare providers and community organizations. For example, a clinic may refer a resident for nutrition support, a school may identify a student needing behavioral health evaluation, or a pharmacy may connect a resident to medication support.

Q56. What is a social care referral (SCR) platform?

A SCR platform is a tool that allows users to send, receive, and track referrals for social services (e.g., food, housing, transportation, or utility support). It is a closed-loop referral system that connects people to services outside of healthcare.

Q57. Will the social care referral platform connect with EHRs or be separate?

The SCR platform and electronic health records are intended to work as part of an interoperable system enabling secure data exchange and patient monitoring across providers.

Community institutions will be supported through onboarding, training, and technical support to implement these systems.

Q58. Will all Missouri providers be on the same EHR platform?

No. The Digital Backbone is intended to support interoperability across providers and systems, not require every provider to use one state-run EHR platform. EHR modernization support may include targeted upgrades and limited new installations where appropriate.

Q59. How is technology being chosen, and how can organizations provide input?

The recent digital readiness survey sent to Missouri providers is informing technological decisions, determining where needs are greatest among partners across rural Missouri. Organizations should follow official communications tied to survey results, follow-up opportunities, and procurement announcements.

Q60. Will cybersecurity be part of the program?

Yes. Cybersecurity was included in the recent digital readiness survey and enhancements / technical assistance will be provided to organizations with identified gaps to support stronger cybersecurity infrastructure.

Q61. Will there be statewide measures for improved outcomes?

Yes. There will be statewide performance metrics tracked across Hubs and RCNs across access, outcome, and sustainability impact areas. See the Hub Anchor application for further details.

Q62. How will program performance be monitored?

Hubs will track metrics following state-standardized reporting requirements and metric definitions, including referrals, follow-ups, ED visits, hospitalizations, disease control, maternal outcomes, behavioral health access, telehealth visits, and other required measures.

Dashboards will be reviewed regularly with the Hub Leadership board, RCN, and RHTO.

Next steps

Q63. When will rural residents begin to see services or new programs?

We expect Hubs to begin launching the first phase of new programs throughout Q3 – Q4 2026. As the program continues to scale in 2027 and beyond, new services, new initiatives, and improved care coordination will be launched to improve healthcare outcomes and access for rural residents across Missouri.

Q64. What should interested organizations do now?

Interested organizations should:

- Review the official Hub boundary map
- Review the Hub Anchor application
- Connect with local providers and community partners to align on Anchor organizations and begin assessing local needs to prioritize targeted programs supporting local residents
- Prepare to participate in Hub Leadership boards, working groups, referral workflows, onboarding, and other training
- Follow official RHTP communications and procurement announcements through the website and email linked at the top of the FAQ document